

Letter of instruction — psychiatric report

Dr Ravikumar Bangalore Krishnaiah, Consultant Psychiatrist, Mind and Memory Service

From: Cory Lea Shepherd · WC/2024/227, Queensland Industrial Relations Commission · [DATE]

■ **DRAFT.** Bracketed items require confirmation before sending.

PART A — THE ATTACHMENTS

Four documents accompany this letter. **Three were written by the other parties to my matter.**

No.	Document	Author	Status
1	Response to Notice to Admit Facts , 18 February 2026 — the Regulator's admissions in this appeal.	The Regulator	Assumed fact
2	Letter of the Chief Executive , Metro South Hospital and Health Service, 5 June 2026 (ref K-LM26/729), to the Commission.	The employer's Chief Executive	Assumed fact
3	Role description — Administration Officer, Switchboard Services (AO3), Logan Hospital.	The employer	Assumed fact
4	The clinical record — your file for me from 24 October 2024; my general-practice records (Our Medical Ashmore), including the entries of 26 October 2022 and 16 November 2023; the Work Capacity Certificate of Dr Hawes; my certificate of capacity of 3 July 2026.	Clinicians	Clinical record

PART B — THE BASIS ON WHICH I ASK YOU TO WRITE

I ask you to assume, for the purpose of your opinion, **the facts recorded in Attachments 1 to 3**. Attachment 1 records facts admitted by the respondent in the proceeding; Attachments 2 and 3 are the employer's own documents.

My own account of the workplace is provided as clinical context only, and is not the basis on which I ask you to reason. Where your history from me differs from any fact in Attachments 1 to 3, please say so expressly rather than resolve the difference. Attachment 4 is the clinical record, for your clinical review.

Please do not adopt, or treat as established, any conclusion, characterisation or finding of any other decision-maker. **Your opinion is yours alone.** I do not ask you to express any view on whether any management action was reasonable; that is a question for the Commission.

Dates. So that the sequence is available to you from a source other than my own account, the following dates are taken from the Regulator's amended List of Documents dated 14 August 2026, by its item number.

Item	Date	Document
46	1 July 2024	Text, WorkCover to me, noting the claim registered
7	1 July 2024 11 August 2024 8 September 2024	Workers' compensation medical certificate — Dr Peter Hawes
8	7 August 2024	Workers' compensation medical certificate — Dr Ki Pang
2	13 September 2024	WorkCover Queensland reasons for decision
3	16 September 2024	Application for review
4	24 October 2024	Review Unit reasons for decision
9	24 October 2024	Email, Dr Krishnaiah — noting injury and medication
5	26 November 2024	Notice of Appeal
10	13 February 2025	Report of Mind and Memory Service
11	Various	Practice records — Our Medical Ashmore

PART C — THE MATTERS IN ISSUE

You have been served with a Notice of Non-Party Disclosure in this proceeding. That notice states the matters in issue as follows:

- “1. Did Mr Shepherd sustain a personal injury*
- 2. Did the personal injury arise out of or in the course of Mr Shepherd's employment*
- 3. Was Mr Shepherd's employment a significant contributing factor to the injury”*

I ask you to address those three matters, in that order. My employer has separately asked for medical information about my capacity for work; **those matters are taken up within question 3 below**, so that one report answers both.

MATTER 1 — DID MR SHEPHERD SUSTAIN A PERSONAL INJURY?

- 1.1** The diagnosis; the classificatory framework applied and the code; the specific diagnostic criteria met and when each was met; and the date of onset as best the clinical record establishes it.
- 1.2** The differential diagnoses considered, and your reasons for including or excluding each.
- 1.3** My psychiatric background — the entries of 26 October 2022 referring to anxiety and ADHD, any earlier stimulant use, and the entry of 16 November 2023. Whether there was any relevant pre-existing condition; and if so, whether the matters at Attachments 1 to 3 aggravated it, and to what extent.
- 1.4** Whether any premorbid personality features bear on the diagnosis, and if so how — from your own current clinical assessment. I do not ask you to adopt or repeat any earlier characterisation.
- 1.5** A complete list of every document you reviewed and every person you saw, with the dates and duration of each examination.

MATTER 2 — DID THE INJURY ARISE OUT OF, OR IN THE COURSE OF, THE EMPLOYMENT?

2.1 The conditions of the work. Attachment 3 records that the position requires continuous shift work across the full 24-hour period, seven days a week; participation in the Emergency Response process, distributing emergency notifications in accordance with code procedures and strictly adhering to protocols and timeframes; maintaining call queues to a minimum at all times; multitasking and operating under pressure with high volume call traffic; exercising judgement where precedent has not been set and procedures are not defined; and working with limited supervision. Attachment 1 records that the accuracy of Switchboard information is critical to clinical handover and patient safety.

From a clinical perspective, whether and how conditions of that kind — the level of demand, the degree of control, and the adequacy of support — can contribute to a condition of the kind you have diagnosed.

2.2 Fatigue and its management. Attachment 2 records that no fatigue risk assessment applied to the position; that fatigue risk management was implemented at the Switchboard only after 30 June 2024; that complaints were managed solely by email or verbally; and that no consequential changes to operating procedures were made over the period. Whether, and if so how, sustained unmanaged fatigue in work of that kind bears on the condition you have diagnosed.

2.3 The rostering of 17 to 18 March 2024. Attachment 1 records a rostered break of 7 hours against a 10-hour minimum, described as a rostering error accidentally made. Attachment 2 records that emergency (MET) calls were recorded on those shifts, and that leave was taken on 19 March 2024. What clinical significance, if any, do you attach to that sequence?

MATTER 3 — WAS THE EMPLOYMENT A SIGNIFICANT CONTRIBUTING FACTOR?

3.1 Causation. On the assumed facts, whether my employment was a **significant contributing factor** to the injury with onset on 18 June 2024. Please give your clinical reasoning, and distinguish between the cause of onset and any factors bearing only on the subsequent course.

3.2 The interval. The rostering matters at 2.3 occurred in March 2024 and onset is pleaded at 18 June 2024. Is that interval consistent with, or inconsistent with, your formulation — and if consistent, by what mechanism? Separately, what significance, if any, attaches to the interval between onset and first presentation, the Work Capacity Certificate recording first attendance for this injury on 1 July 2024?

3.3 Other factors, placed in time. Please identify any non-employment factors relevant to (a) onset at or about 18 June 2024 and (b) the subsequent course. Your report of 13 February 2025 referred to multiple life stressors including relationship breakdown, job loss and bereavement.

Your clinical relationship with me began on 24 October 2024, and my partner attended that consultation with me. Each of the matters above arose while I was under your care and is recorded in your own file. I ask you:

(a) to place each factor in time **from your own records rather than by assumption**;

(b) to state whether, **as at the date of your diagnosis**, my relationship was a protective factor or an adverse one; and

(c) for each factor, whether in your opinion it was **independent of the employment matters, or a consequence of them** — noting that your report of 13 February 2025 records that my pay was *“withheld or delayed for up to five months at a time, leading to significant financial stress and strain on his relationship”*.

Capacity, restrictions and adjustments — the matters my employer has asked about. My certificate of capacity of 3 July 2026 certifies me fit for my substantive role with adjustments; I ask only that your opinion be your own.

3.4 Prognosis and current capacity. Your prognosis; and, distinguishing (a) my capacity to perform the substantive role **with reasonable adjustments** from (b) any current incapacity. If there is current incapacity, its cause — the condition itself, the consequences of my exclusion from the workplace since 3 July 2026, or other factors.

3.5 The adjustments, in functional terms. The adjustments you consider clinically necessary, the clinical basis for each, and the anticipated duration and review date — addressing, among anything else you consider relevant: predictability of rostering; adequate recovery between shifts; sustained concurrent emergency-code load; hours of work; and reporting arrangements.

3.6 Inherent requirements. Whether, **with the adjustments at 3.5**, I am able to fulfil the requirements of the position described at Attachment 3; and whether those adjustments are ordinary and available in a workplace of that kind.

3.7 Exacerbating conditions. Whether there are specific tasks, situations or environments that may exacerbate the condition or its symptoms.

3.8 Complaint handling. If you consider a restriction on complaint handling clinically necessary, what activities it encompasses in functional terms — receiving, documenting, redirecting or resolving complaints, or all complaint-related interaction.

3.9 Working memory. Your report of 13 February 2025 records that my working memory is affected under stress. What that means in functional terms in a workplace, including how it may affect performance of the duties at Attachment 3 and the circumstances likely to give rise to the difficulty.

3.10 Foreseeable risk. Whether exposure to the conditions at Attachments 2 and 3, without the adjustments at 3.5, presents a foreseeable risk to my health or safety; and if so, what controls you consider medically necessary to manage it.

PART D — WHAT I DO NOT ASK, AND THE FORM OF THE REPORT

I do not ask you to express a view on whether my employer is able to accommodate the adjustments you recommend, or on my capacity to work under any particular reporting line or with any particular person. Those are workplace questions, not medical ones. If you are asked either question, I ask that you say so.

Please reason from the assumed facts to your conclusions rather than assert conclusions, and state where the material is insufficient for you to express an opinion rather than qualifying an opinion you would not otherwise give.

Please state expressly the material on which your opinion as to causation rests, and whether that opinion depends on my account of the workplace matters.

The report is prepared for use in proceedings in the Queensland Industrial Relations Commission and may be provided to my employer. [CONFIRM: the standard practice footer restricting use must not appear on this report.]

[CONFIRM: fee; and whether Review Decision 69983 was provided at the consultation of 12 August 2026 — if it was, an express direction not to adopt any finding in it, and a question whether the opinion would differ if it were disregarded, must be added.]